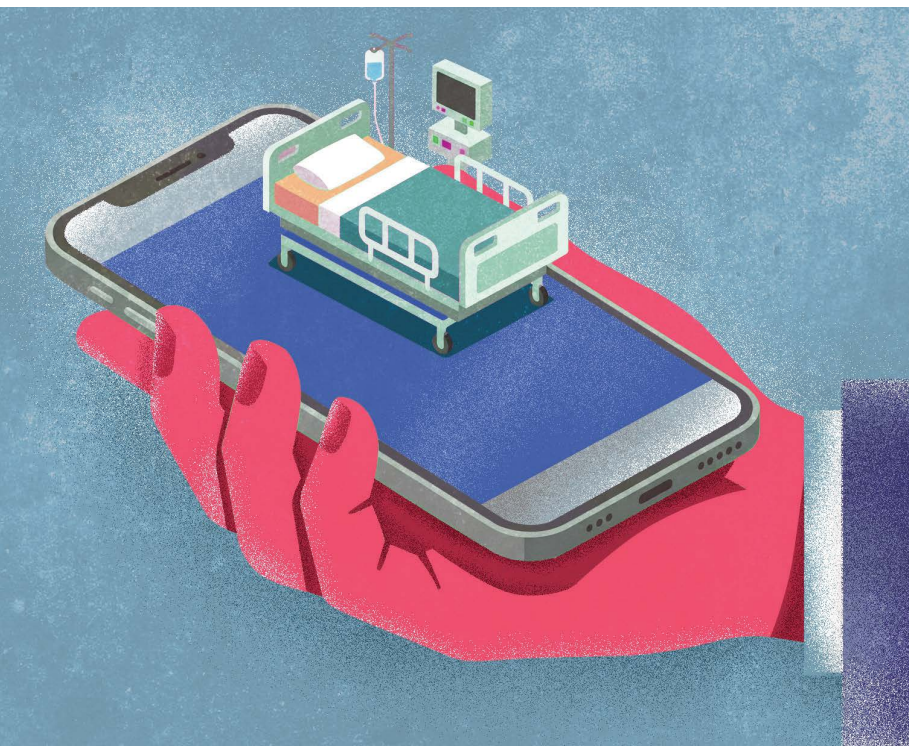




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TECH AS AN ENABLER

Health financing must also evolve. While publicly financed insurance has reduced the financial burden of hospitalisation for many families, out-of-pocket expenditure on outpatient consultations, diagnostics and medicines remains high. Greater investment in primary care, uninterrupted supply of essential diagnostics and medicines, prevention and health promotion will not only improve health outcomes but also reduce avoidable hospital admissions. Financing mechanisms should increasingly encourage quality, continuity and better health outcomes, alongside sustained public investment and greater flexibility for states to address local priorities. Technology can support this transformation if used wisely. Digital health records, telemedicine and clinical decision-support systems

have the potential to improve continuity of care, strengthen disease surveillance and support frontline health workers. But technology is an enabler, not an end.

Its value lies in making healthcare more accessible, more coordinated and more responsive to people's needs. Ultimately, UHC cannot be achieved by the health sector alone. Air pollution, climate change, unhealthy diets, education, sanitation and other social and commercial determinants of health increasingly shape disease patterns and health outcomes. Addressing these challenges requires coordinated action across sectors and stronger engagement with communities, who should be recognised not merely as beneficiaries of health services but as partners in shaping them. *Jan Swasthya* committees, *Gram Vikas Sabhas* and *Panchayats* must all play a major role

in ensuring that quality health care is provided and social determinants of health addressed.

Every state must have a public health cadre—this was envisioned in the National Health Policy 2017 but not realised. The health care sector can also be a big employer of people and an opportunity to address the lack of jobs for our youth. We will need more lab technicians, pharmacists, occupational therapists, optometrists, palliative care givers, and emergency care technicians as public provisioning of health care expands. We also need more public health entomologists and nutritionists at the district level to address the growing burden of vector-borne diseases and nutritional problems (including obesity).

The challenge now is to build on the progress already made. Sustained investment in primary healthcare, greater emphasis on prevention, effective stewardship by government and accountable partnerships with the private sector will be essential to creating a health system that is equitable, resilient and responsive to people's needs. If these elements come together, India's journey towards *Viksit Bharat* can become a powerful example of how better health drives inclusive growth, social equity and national development. **BT**

The author is the Principal Advisor to the National Tuberculosis Elimination Programme, Ministry of Health and Family Welfare and the Chair of M. S. Swaminathan Research Foundation (MSSRF). She was the first Chief Scientist of the World Health Organisation (WHO). Previously, she was the Director General of the Indian Council of Medical Research (ICMR), Ministry of Health and Family Welfare, Government of India (2015-17).

Roubitha David has also contributed to this column. Views are personal